Health Care Delivery complete notes

health records and information technology (Kenya Medical Training College)

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UNIT CODE: DHR2211 HEALTH CARE DELIVERY

INTRODUCTION

***A** health system* consists of all organizations, institutions, people and actions whose primary intent is to promote, restore or maintain health" *(WHO, 2007)*

***Broadly a health* system** includes the **resources, actors** and institutions related to the financing, regulation and provision of health actions.

A HEALTH SYSTEM may be described as

A set of cultural beliefs about health and illness that forms the basis for health-seeking and Health-promoting behavior; The Institutional arrangements within which that behavior occurs; and the socioeconomic/political/physical context for those beliefs and institutions.

In short, it consists of what people believe and know about health and illness and what they do to remain healthy and cure diseases. Beliefs and action are usually closely connected. For example, if in a society people perceive evil ancestor-spirits as a cause of disease, there will be specialists and rituals to appease those spirits. If they see germs as the cause, they will look for modern (biomedical) health care. If biomedical health care is a recent introduction, people may accept services, but the beliefs and knowledge to support this behavior may not have been fully developed.

Health workers therefore should be aware of the indigenous explanations for illness, so that "biomedical" explanations can be adapted to these more deeply rooted indigenous concepts. Institutional arrangements within which health behavior occurs encompass more than the delivery of medical care through governmental health services. They include all individuals, groups, and institutions that directly or indirectly contribute to health. It may differ from society to society but usually covers the following;

1. **The individual, family, and community**

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The individual, family, and community assume a vital responsibility for health promotion as well as for the curative care of its members. In any society as much as 70-90% of all curative Activities may take place within this network. Several studies, carried out in Western and non-Western societies, support this statement (Kleinman 1978).

2. Health care services

There are health care services in the public (government) sector as well as the private sector.

Public sector health care services

These include: Health workers at village level, mobile health teams, rural health clinics, and their outreach services (e.g., midwifery, sanitation, nutrition, malaria control, etc.); Health centers, urban clinics, district hospitals, and large multispecialty hospitals with their various support services such as laboratory, radiology, pharmaceuticals, etc.; and Institutions responsible for health personnel, health financing, and physical infrastructure.

Private sector health-care services

This sector may include: Folk (or traditional) medicine with traditional birth attendants, herbalists, and diviners, who may identify natural or supernatural causes of diseases and treat them accordingly; The large non-Western professionalized healing systems (Ayurvedic, Chinese, Yunani, homeopathic, chiropractic, etc.). In some societies these belong to the public sector; Private "modern" medical practice, legal or illegal; the pharmaceutical sector (private or parastatal); and Nongovernmental health care (churches, Red Cross, etc.).

The relative importance of these components varies in different societies.

3. Health-related sectors

These include, for example:

- Agriculture and food distribution;
- Education (formal and non-formal);
- Water and sanitation; and
- Transport and communication.

All these sectors contribute to health, either directly or indirectly

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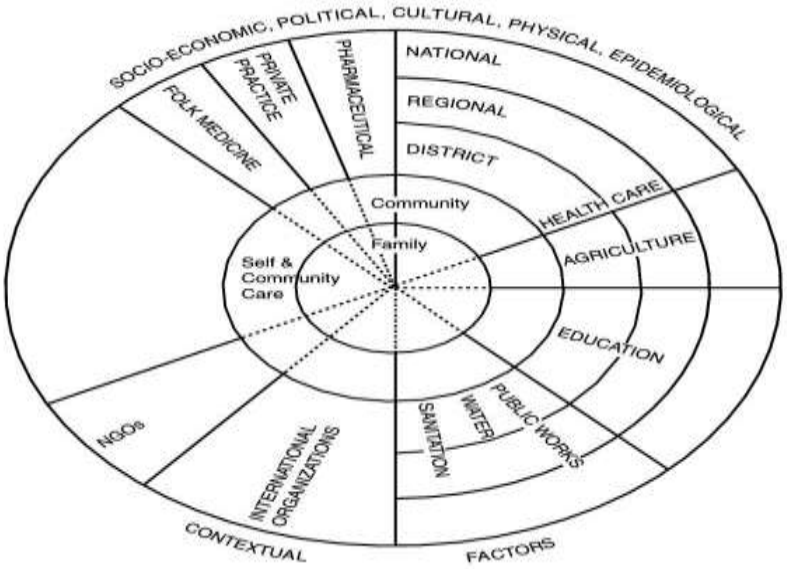
4. The international sector, including bilateral and multilateral donor agencies (UNICEF, WHO, etc.) that may support health as well as development activities.

Health services may not always be their first choice. In many countries, efforts are being made to increase coordination between the various public services, and between governmental services, NGOs, and other healing systems to promote health.

Broad definition of health systems



Health system has elements which are;

- **inputs** (drugs, funds, infrastructure, and equipment)
- **processes** (clinical services, management services, technological process)
- **Outputs** (health: persons treated, recovered or died: responsiveness or fairness).

Health system differs from *a **healthcare system***, which is a means of organized social response to the health conditions of the population. It is narrow than a health system and is often described in terms of the levels of healthcare and organizational structure of the ministry responsible for health in most countries.

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HS Framework: Goals, Objectives.

- **Health System goal**: *to improve health and health equity, in ways that are responsive, financially fair, and make the best, or most efficient, use of available resources.* (legitimacy and fairness)

- **Health System Objectives**: Improve *access*, and *quality* of services, achieve better *coverage* for services without compromising *safety*

- **Health System building blocks**: there are Six investment areas aims to improve coverage of services through attaining desired access to interventions, without compromising quality and safety. The six pillars of the health system include:

    1. **Service delivery;**
    2. **Health workforce.**
    3. **Health information;**
    4. **Medical products, vaccines and technologies;**
    5. **Health financing**
    6. **Leadership and governance (stewardship).**

The building blocks need to be well functioning if the health system has to produce the expected health actions

Principles of a health system

- **People-centered -** Equity and fairness
- **Results-oriented -** Quality management system for continual quality improvement
- **Evidence-based -** Technocrats, academicians, politicians, community/local context and change
- **Community-driven-** Leadership, governance accountability, transparency and sustainability
- **Context-specific -**, context is synonymous with resource-constrained environment

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- **Ethically sound** – Human rights and dignity, safety for the client, community and environment
- **Systems thinking-** Holistic view of the health system

Characteristics of a well-functioning Health System.

1. Good **health services** are those which deliver *effective, safe, quality* personal and non-personal health interventions to those that *need them, when and where needed, with minimum waste* of resources.
2. A well-performing **health workforce** is one that works in ways that are responsive, fair and efficient to achieve the best health outcomes possible, given available resources and circumstances (i.e., there are *sufficient staff, fairly distributed; they are competent, responsive and productive*).
3. A well-functioning **health information** system is one that ensures the *production, analysis, dissemination and use of reliable and timely* information on health determinants, health system performance and health status.
4. A well-functioning health system ensures equitable access to essential **medical products and technologies** of assured quality, safety, efficacy and cost-effectiveness, and their scientifically sound and cost-effective use.
5. A good **health financing** system raises adequate funds for health, in ways that ensure people can use needed services, and are *protected from financial catastrophe or impoverishment associated with having to pay* for them. It provides incentives for providers and users to be efficient.
6. **Leadership and governance** involve ensuring *strategic policy frameworks* exist and are combined with *effective oversight*, *coalition-bui*lding, *regulation,* attention to *system-design* and *accountabilit*y

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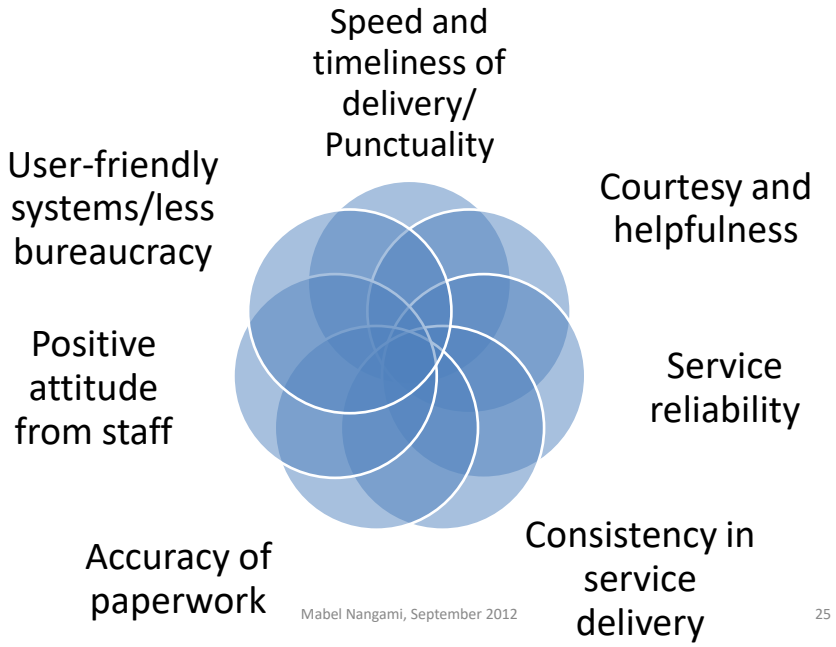
Characteristics of a Responsive Health System



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The Nine Principles of Health Care Delivery

Affecting intermediate outcomes:

1. Access
2. Coverage
3. Quality
4. Safety

Affecting final outcomes:

5. Equity
6. Efficiency
7. Effectiveness
8. Client Centeredness
9. Sustainability

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Organization of Health Systems

- The **management structure** of the health system is made up of officials who provide stewardship through policy, strategic and operational directives that they provide to the different actors in the system, and also to manage resources for health to ensure that services are provided to those who need them.
- The **governance structures** of the health system are created to provide an oversight role to the management structures. They are supposed to provide voice for the citizens to ensure that people's rights of access to quality health care are respected.
- **Coordination/Partnership structures** provide opportunity for stakeholders to share experience, expertise and lessons learnt. They form the basis for harmonization and alignment thereby minimizing duplication of efforts by stakeholders.

STRUCTURE OF HEALTH SERVICES

- In Kenya, health services are now organized based on:
  - Type
  - Target group (Cohorts)
  - Level
- **By Type:** The KHSSP III 2012–2017 defines the Kenya Essential Package for Health (KEPH):

I. Promotive services i.e., Health Education, diet, giving ARVs

II. Preventive services:
   - vaccinations i.e., polio, BCG
   - Giving condoms, Prep
   - Counselling
   - Giving Various Intervention i.e., Mosquito Nets
   - Giving information about Injury Prevention
   - PMCTC

III. Curative services;
   - Offer antibiotics, antifungal, antiprotozoal drugs
   - Surgical services /Operation.

IV. Rehabilitative services
   - Physiotherapy
   - Drug addiction rehabilitation.

- **BY Cohorts (target)**

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KEPH revised Cohorts represent the life-cycle approach to health service management. Each age group has special needs that relate to the development phase they are passing through. Services based on 5 life cohorts and cross-cutting category, encompassing the expectations of persons in Kenya.

- *Pregnancy and the new born (up to 28 days)*
- *Early childhood (29 days – 59 months)*
- *Late childhood and youth (5 – 19 years)*
- *Adulthood (20 – 59 years)*
- *Elderly (60 years and over)*
- *Cross cutting interventions (All cohorts)*

I. Pregnancy and newborn cohorts

Goes up to 28 days after birth and before

- ✓ Pregnancy services
- ✓ PMCTC
- ✓ Antenatal and postnatal
- ✓ Supplements i.e., Iron supplements
- ✓ Tetanus Vaccinations
- ✓ CS – Cross examination of wounds

Newborn services; Vaccinations, BCG at birth, Pentavalent at 4 weeks

II. Early childhood cohort

From 29days to 5years

- ✓ Pentavalent vaccines
- ✓ Measle vaccination
- ✓ Polio vaccinations
- ✓ Deworming

III. Late childhood & Youth cohort (5-19years)

Screening, vaccinations, counselling, health education.

- ✓ Sex education
- ✓ Deworming
- ✓ Injury preventions

IV. Adulthood cohort (20-59 yrs.)

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- ✓ Sex education
- ✓ Family planning services
- ✓ Injury prevention
- ✓ Screening
- ✓ Health education
- ✓ Nutritional services

V. Elderly Cohort (20-59 yrs.)

- ✓ Screening
- ✓ Rehabilitative services
- ✓ Curative services
- ✓ Injury prevention.

- **By Tiers (Levels).**

KEPH services defined for each of the 6 cohorts for the newly defined 4 tiers of the Health System (KHSSP III **2012- 2017)** which are interlinked

- o Tier 1: Community level
- o Tier 2: Primary Care level (Health Centers/Dispensary)
- o Tier 3: County level (District /Sub district hospitals)
- o Tier 4: National level (Teaching/referral/specialist HF)

Tier 1: Community level

This form of health care service delivery, we have community health worker, community volunteers, CORPS & CHEWs

They facilitate individuals' households and community carries out appropriate health behaviors. They also agreed health services, recognize sign and symptoms of a disease, also refer to a health facility, facilitate community diagnosis

Tier 2: Primary Care level (Health Centers/Dispensary)

Dispensaries, Clinics, Maternity Home, Health Care Centers

Functions

- Disease prevention and health promotion services i.e., immunization
- Health promotion services – health education, awareness.
- Basics of patient diagnostics.
- Medical/ Minor surgery.

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- Rehabilitative services i.e., Counselling, physiotherapy Services.
- Inpatient Services for emergency clients awaiting referral and client observation.
- Normal delivery services.
- Facilitate referral of clients from communities and to other higher tiers.

Tier 3: County level (District /Sub district hospitals)

Former district and sub district hospital which include NGO and private hospital i.e., Thika Level (V) Hospital and Ruiru Hospital.

Functions

- Comprehensive patient diagnostic
- Surgical and rehabilitative care
- Reproductive health care services:
    - Safe motherhood
    - Breast feeding
    - Family planning
    - Parental mother to child transmission
- Specialized outpatient services
- Facilitate and manage referrals from lower levels

Tier 4: National level (Teaching/referral/specialist HF)

Made up of:

- Provincial General Hospital (PGH)
- National Referral Facilities i.e., KNH
- Specialized Health Care i.e., Mathari Hospital
- National Spinal & Injury Centre i.e., Texas Cancer Centre.

Functions

- Offer highly specialized health care
- Training
- Research services on issues of national importance i.e., UNITED

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Types of services provided;

V. Promotive services i.e., Health Education, diet, giving ARVs

VI. Preventive services:
- vaccinations i.e., polio, BCG
- Giving condoms, Prep
- Counselling
- Giving Various Intervention i.e., Mosquito Nets
- Giving information about Injury Prevention
- PMCTC

VII. Curative services;
- Offer antibiotics, antifungal, antiprotozoal drugs
- Surgical services /Operation.

VIII. Rehabilitative services
- Physiotherapy
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The needs and expectations of health information system

Mandate of health information system

- To collect, process, analyze and disseminate health information to users

Vision of the Health Information System

- Be a center of excellent for quality health and health related data and information

Mission of the Health Information System

- To provide timely, reliable and accessible quality health information for evidence-based decision making to promote the health of the nation

Policy goal

- This policy seeks to enhance availability of comprehensive quality health and health related data and information for evidence-based decision making. It seeks to address such issues as partnership in data collection and information sharing, guidelines and data processing, and data warehousing as well as instituting standardized mandatory reporting by all care providers and quality in data management in the health sector

Policy Objectives

a) Promote one health information system in Kenya upon which all shall be committed to.
b) Promote use of health information for evidence-based decision making, promote accountability and empower citizens to make healthy choices
c) Promote collection of sufficient, relevant, reliable and quality health statistical data pertaining to the health status of the nation, health services coverage and utilization.

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d) Promote and encourage production and dissemination of timely, easily understood health and health related information for evidence-based decision making by managers at various managerial levels within the health sector
e) Enhance closer co-operation between producers and users of health-related data and information through regular meetings, seminars, training and publications.
f) Promote reporting by all health-related statistical constituencies through the use of standardized data collection and reporting tools

The Health Services objective for the Kenya Health Policy is to attain universal coverage with critical services that positively contribute to the realization of the overall policy goal. Six policy objectives, therefore, are defined, which address the current situation – each with specific strategies for focus to enable attaining of the policy objective.

1. Eliminate communicable conditions: This it aims to achieve by forcing down the burden of communicable diseases, till they are not of major public health concern.
2. Halt, and reverse the rising burden of non communicable conditions. This it aims to achieve by ensuring clear strategies for implementation to address all the identified non communicable conditions in the country.
3. Reduce the burden of violence and injuries. This it aims to achieve by directly putting in place strategies that address each of the causes of injuries and violence at the time.
4. Provide essential health care. These shall be medical services that are affordable, equitable, accessible and responsive to client needs.
5. Minimize exposure to health risk factors. This it aims to achieve by strengthening the health promoting interventions, which address risk factors to health, plus facilitating use of products and services that lead to healthy behaviors in the population.
6. Strengthen collaboration with health related sectors. This it aims to achieve by adopting a 'Health in all Policies' approach, which ensures the Health Sector interacts with and influences design implementation and monitoring processes in all health related sector actions.

Strategic Objective 1: Eliminate Communicable Conditions

Through this first strategic objective, the sector aims to force down -the burden of communicable diseases, to a level that they are not of major public health concern. In the medium term, the priority strategies include:

i. Increase access of the population to key interventions addressing communicable conditions causing the highest burden of ill health and death
ii. Ensure communicable disease prevention interventions directly addressing marginalized and indigent populations
iii. Enhance comprehensive control of communicable diseases by designing and applying integrated health service provision tools, mechanisms and processes

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Strategic Objective 2: Halt, and Reverse rising burden on Non Communicable Conditions

This second strategic objective will focus on ensuring efforts are initiated to prevent a rise in the burden of key non communicable conditions affecting persons in the country. In the medium term, the sector will focus on:

i. Providing prevention activities addressing the major non communicable conditions
ii. Put in place interventions directly addressing marginalized and indigent populations affected by non-communicable conditions
iii. Integrating health service provision tools, mechanisms and processes for NCD's
iv. Establish screening programs in health facilities for major non communicable condition

Strategic Objective 3: Reduce the burden of violence and injuries

The third strategic objective will focus on managing the burden due to violence and injuries affecting persons in the country. In the medium term, the sector will focus on:

i. Make available corrective and intersectoral preventive interventions to address causes of injuries and violence
ii. Scaling up access to quality emergency care (curative and rehabilitative) that mitigates effects of injuries and violence
iii. Put in place interventions directly addressing marginalized and indigent populations affected by injuries and violence
iv. Scale up physical, and psychosocial rehabilitation services to address long term effects of violence and injuries

Strategic Objective 4: Provide essential health services

These shall be affordable, equitable, accessible and responsive to client needs. This will be achieved by strengthening the planning and monitoring processes relating to health care provision, to ensure demand driven priorities are efficiently and effectively provided to the populations, based on their expressed needs. The priority policy strategies to achieve this are:

i. Scale up physical access to person centered health care, with local solutions designed to fore hard to reach, or vulnerable populations
ii. Ensure provision of quality health care, as defined technically, and by users
iii. Avail free access to trauma care, critical care, and emergency care and disaster care services.
iv. Initiate efforts to promote medical tourism as a means to ensure high quality care availability in the Country

Strategic Objective 5: Minimize exposure to health risk factors

The objective is focused on putting in place appropriate Health Promotion interventions that will address risk factors to health. These include:

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i. Reduction in unsafe sexual practices, particularly amongst targeted groups
ii. Mitigate the negative health, social and economic impact resulting from the excessive consumption and adulteration of alcoholic products
iii. Reduce the prevalence of tobacco use and exposure to tobacco smoke and other harmful addictive substances
iv. Institute population-based, multi sectoral, multidisciplinary, and culturally relevant approaches to promoting physical activity and healthy diets
v. Strengthen mechanisms for screening and management of conditions arising from health risk factors at all levels.
vi. Increase collaboration with research based organizations and institutions

Strategic Objective 6: Strengthen Collaboration with health related sectors

As highlighted in the National Health Policy, this strategic objective highlights the key services and interventions that have a secondary effect on health. These health related interventions are in the following sectors (but not limited to).

i. Economy and employment: Ensure work and stable employment and entrepreneur opportunities for all people across different socio economic groups
ii. Security and justice: Have fair justice systems, particularly in managing access to food, water & sanitation, housing, work opportunities, and other determinants of wellbeing
iii. Education and early life: Support education attainment of both women and men to promote abilities to address challenges relating to health
iv. Agriculture and food: Incorporation of considerations of health in safe food production systems, manufacturing, marketing and distribution
v. Nutrition: Ensure adequate nutrition for the whole population, through avoiding and managing over, or under nutrition
vi. Infrastructure, planning and transport: Optimal planning of health impacts for roads, transport and housing investments, to facilitate efficient movements of people, goods and services relating to health
vii. Environments and sustainability: Influence population consumption patterns of natural resources in a manner that minimizes their impact on health
viii. Housing: Promote housing designs and infrastructure planning that take into account health and wellbeing
ix. Land and culture: Strengthening access to land, and other culturally important resources by particularly women
x. Population: Manage population growth and urbanization implications
xi. Gender and vulnerable populations: Strengthen identification of special groups and their needs so as to increase equitable access to health care services

Regulatory agencies

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There are eight health professionals regulatory agencies established through acts of Parliament to represent individual professions. They include,

I. the Nursing Council of Kenya (NCK),
II. Medical Practitioners and Dentist Board (MPDB),
III. Clinical Officers Council (COC),
IV. Kenya Medical Laboratory Technicians and Technologists Board (KMLTB),
V. Pharmacy and Poisons Board (PPB),
VI. Public Health Officers and Technicians Council (PHOTC),
VII. Radiation Protection Board (RPB) and
VIII. Kenya Nutritionists and Dieticians Institute (KNDI).

Regulatory functions

Kenya health professionals' regulatory boards and councils regulate the training and practice of health professionals, which can include professionals and training institutions as well as health facilities. Although functions vary somewhat among the various agencies, the following are the general functions of most regulatory agencies.

I. Accreditation of training institutions

All training institutions, whether public, private or faith-based, are required to be approved by their respective health professional regulatory agencies. The process of accreditation of training institutions begins with the established institution applying for accreditation to the regulatory agency, followed by inspection of the institution, and review of the curriculum, after which the regulatory agency makes a decision on whether or not to approve the institution. After approval, institutions are allowed to admit students. Thereafter, inspection of institutions is a random continuous activity to promote quality training of health professionals.

II. New student enrolment

Upon accreditation, institutions are required to submit data on the enrolment of new students, to enable the regulatory agencies to keep track of their performance in a process commonly referred to as "indexing." When students begin their training, key demographic data are collected, including county of origin, secondary school, date of birth and gender. Regulatory agencies use data provided on each student to ensure that the students meet the minimum qualifications set by the agency for entry into the various training programs. Regulatory agencies assign each student an index number, which serves as a unique identifier for that individual through graduation and into their career.

III. Internship and licensing examinations

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After students show proof of completing the approved training, regulatory agencies recommend certain health professional students for internship posting by the Ministry of Health, whereas other cadres proceed directly to licensure examinations.

IV. Licensing and registration

Students who have passed their national qualifying exams (when required) and the stipulated internship period (when required) are allowed to apply for licensing and registration.

V. Regulation of registered professionals

Regulation of the health workforce is a continuous quality control measure. Regulatory agencies maintain and update registers of active health professionals.

VI. Disciplinary actions

The regulatory agencies are responsible for investigating disciplinary cases and administering disciplinary actions against health professionals who are found culpable.

VII. Immigration and out-migration

Regulatory agencies oversee the immigration and out-migration of health professionals in Kenya. They are responsible for determining suitability of foreign-trained health professionals who apply for licensure to practice in Kenya. In that regard, regulatory agencies administer various competence tests, additional training, and internship to foreigntrained professionals to match Kenya's quality requirement.

VIII. Health facility inspections

The regulatory agencies inspect private and public health facilities to guarantee quality healthcare. They conduct periodic joint inspections of private healthcare facilities in conjunction with the Ministry of Health to ensure quality healthcare standards are achieved

Health care delivery system

The main function of health care delivery and quality of care is to increase the coverage and quality of promoting preventive and curative activities.

If a better performing health system is to be attained, adequate and motivated personnel, availability of medical supplied and sustainable financial resources are conditions to achieve such as objective.

The main purpose of the health service is to give a comprehensive and integrated primary health care (Health Extension Package) at the community level. The approach will be to emphasize on the preventive and promotive aspect of health care without neglecting essential curative services.

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Establishment of a Professional Regulatory Board

The Health Information Management (HIM) profession in Kenya has been created because of a need for accurate, complete data regarding the care and treatment of patients and the production of timely information for evidence based decision making within the health sector. HIM professionals like their other colleagues within the health sector need to be regulated in order to ensure that they operate professionally. Regulatory mechanisms for Health Records and Information Management, like any other health profession, shall include activities like state licensure of practitioners, accreditation of programmes, certification of practitioners, and legislation that governs practice. This calls for the establishment of a Professional Regulatory Board which shall be responsible for:

a) The development of HIM professionals through setting up educational standards for quality educational programs and providing recognition for the educational programs that meet those standards
b) Certification of educational programmes for HIM profession, Research and Advocacy
c) Regulation of entry into health information management profession by ensuring HIS is staffed by qualified personnel

Health Planning

Definition

Planning - is the combination of compiling and analyzing, information dreaming up ideas, using logic and imagination and judgment in order to come to a decision about what should be done.
Health Planning - Is the process of defining community health problems, identifying needs and resources, establishing priority goals, and setting out the administrative action needed to reach those goals.

Features of Planning:

1) A good plan should give
  a) Clear vision/mission, goal and objectives
  b) A clear picture of the tasks to be accomplished
  c) The resources needed to accomplish the task
2) Planning takes place at all levels
  a) planning takes place at any level in health system
  b) planning takes place continually, it is cyclic/spiral process.
  c) planning methods can be applied to:- . A large program at national level eg. Malaria control programme . Small one - at village level eg. Construction of community health post.
3) Planning must be collective undertaking

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a) It requires the participation of:- . Professionals – from health and other sectors . Community/Non government organizations (NGO) . Government/Party

Types of Planning

a) **Strategic planning** –often referred as alocative planning– normally five years or more. Strategic planning is the process of determining what an organization intends to be in the future and how it will get there. It is finding the best future for your organization and the best path to reach that destination.
b) Tactical/operational planning –may be referred to as activity planning. It covers a short period of time medium term –usually one year
Tactical/operational planning is short range planning that emphasizes the current operations of various parts of the organization. Short range is defined as a period of time extending only about one year or less into the future.

Why planning?

The increasing demand for medical and health care services, in the face of limited resources, has brought out the need for careful planning and management of health services

The purpose of planning:

a) To match the limited resources with many problems
b) To eliminate wasteful expenditure and avoid duplication
c) To develop the best course of action to accomplish a defined objective
d) Improved communication among all chains

It's easy for departments at every level to become confused as to what's going on. Both your employees and stakeholders want to ensure that your organization will have a long-lasting future. They want to know where your organization is headed and the steps it takes to get there. Effective planning in healthcare management can help you create clarity and improve communication. Your strategic plan should address the key issues, your organization's vision and goals, and the steps to get there. Your employees and stakeholders will have improved confidence and faith in your organization.

e) **Developing and sharing a vision**

With this in mind, you can make an impact on every level of your organization. Employees will be committed and motivated to help achieve your vision. Stakeholders will have the confidence and clarity they need to make sound financial decisions. Strategic planning for healthcare facilities that's clearly developed, executed, and communicated can help each of your individuals carry out your out vision that can lead to a fulfilling future

f) **Increased employee motivation and engagement**

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Each of your employees wants to be recognized and heard. Being recognized by their leaders can greatly impact their productivity, engagement, and safety management. Every employee wants to have the responsibility to make decisions they know will benefit your organization. This also motivates them to perform above the minimum acceptable standards as outlined in the job description or performance evaluation. Employees won't be motivated to improve themselves for an organization that doesn't state a clear vision or a well-executed game plan.

g) **Transformational leadership and authority**

Transformational leadership is a type of leadership that inspires your employees to work harder and to do better. It incorporates techniques that have been cited in organizational behavior literature. Transformational leaders clearly communicate their organization's vision, believe in their individual employees, and have the abilities to produce high levels of performance. Helping your employees understand how their roles can contribute your organization's mission and vision is a crucial part of strategic management.

h) **Increased team cooperation and collaboration**

Team collaboration and cooperation is an essential component in delivering high-quality healthcare. Employees must work together to make your organization a success. Teamwork is essential for every healthcare industry in order to improve their performance and service. Effective strategic planning models in healthcare can bring your employees together to deliver quality care, great customer service, and increased performance.

Objectives, targets and goals of Health Planning

An important element of planning is the setting of clear cut objectives, targets and goals.

Objective: It is a planned end point of all activities. It is stated in term of measured amount of progress towards goal.

Targets: When the objective is split into discrete activity it becomes target like number of sputum smear collected or Tubectomy done.

Goal: An ultimate desired state towards which the objectives and resources are directed. Goal is not constrained by time, resources nor are they necessarily attainable. They are formulated at top level and they are generally broad for example 'Health for all.

Planning for health involves 3 types of activities

1. Assessment of the current status
2. Identification of the desired state in the future
3. Specification of interventions and other activities to achieve the new desired state.

Planning involves following steps

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- Analysis of health situation
- Establishment of objectives and goals
- Assessment of resources
- Fixing priorities
- Write up of formulate plan
- Programming and implementation
- Evaluation.

Analysis of health situation:

The minimum essential requirement of health planning includes

1. The population its age and sex structure
2. Mortality and morbidity statistics
3. The Epidemiology and geographical distributions of diseases
4. Medical care facilities available such as hospitals, health centres and other private and public health agencies.
5. The man power available
6. Training facilities available
7. Beliefs and attitude of people towards health, disease, its cure and prevention.

The analysis of the above data brings out health problems, needs and demands of the population.

Establishment of goals and objectives

Objectives are the guide for action.

Objectives needed for economical and better performance of programme.

Objectives should be established at all levels at upper level it is more general and lower level it is more detail and specific.

Objectives may be long term and short term. In setting up these objectives time and resources are important factors

Objectives shout be – SMART (Specific, simple • Measurable • Achievable • Relevant • Time bound)

Assessment of resources:

- Man power
- Money
- Materials

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- Skills
- Knowledge and techniques needed or available for the implementation of the health programme.

Fixing priorities

Establish the priorities in the order of magnitude of health problems.

Prioritisation is based on financial constraint, mortality and morbidity data, diseases which can be prevented at low cost, political and community interest.

Once priorities are established alternative plans are formulated and assessed to determine whether they are practicable feasible.

Alternate plan with greater effectiveness are chosen.

Write-up formulated plan

The plan should be complete in all aspect.

The input required for the health programme and the output expected

Cost and time needed for each stage of implementation of the health programme

Working guidance for all those involved in the implementation of the health programme

It must contain a' built in' system of evaluation

Programming and implementation:

Once the plan is approved by policy making authorities programme and implementation are begun.

The effective implantation of the programme depends upon the existence of effective organization.

The main consideration at implementation stage include

1. Defining of the role and task
2. Selection, training, motivation and supervision of the manpower involved
3. Organization and communication
4. The efficiency of the implementing institution

Evaluation:

It is the final step of planning cycle. It should be both concurrent and terminal.

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The purpose of evolution is to assess the achievements of stated objectives, its adequacy, efficiency and its acceptance by the people.

Evaluation measures the degree to which objectives and targets are fulfilled and the quality of the results obtained.

It measures the productivity of the available resource in achieving measured output.

Evaluation makes it possible the reallocation of priorities and of resources on the basis of changing health needs.

Components of the evaluation process:

a) **Relevance**: Relevance relates to the rationale for adopting health policies in terms of their response to social and economic policy; and to having programmes, activities, or services, in terms of their response to essential human needs. For example vaccination against small pox is now irrelevant because the disease is eradicated

b) Adequacy: Adequacy implies that sufficient attention has been paid to certain previously determined course of action, such as the various issues to be considered during broad programming

c) Progress; Progress is concerned with the comparison of actual with schedule activities, the identification of reason for achievements or shortcomings and indications for remedies for any shortcomings. Progress evaluation track of ongoing activities, milestones achieved, personnel matters, supplies and equipment, money spent in relation to budgets allocated.

d) Efficiency: Efficiency is an expression of the relationship between the results obtained from a health programme or activity and the efforts expended in terms of human, financial and other resources, health processes and technology and time.

e) Effectiveness; effectiveness is an expression of the desired effect of a programme, services, institution or support activity in reducing a health problem or improving an unsatisfactory health situation. Thus effectiveness measures the degree of attainment of the predetermined objectives and targets of the programme, services or institution.

f) Impact: Impact is an expression of overall effect of a programme, service or institution on health and related socioeconomic development.Aimed at identifying any necessary change in the direction of health programmes so as to increase their contribution to overall health and socioeconomic development

Basic steps of evaluation

- **Determine what is to be evaluated**

There are types of evaluation

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a) Structure evaluation- evaluation of the resources used in the programme like personnel, money, materials or buildings etc.

b) Process evaluation- the way in various activities of the programme is carried out is evaluated by comparing with the predetermined standard. How the inputs are utilized to produce an output of a service.

c) Outcome evaluation – this is concerned with the end result of the programme. The types and quantities of goals and services produced by the programme.

- Establishment of standards and criteria; Establishment of standards and criteria are necessary to determine how well objectives have to be attained.Structure criteria: physical facilities, personnel and equipments. Process criteria: no. of antenatal visits to be made, no. of blood smear to be collected. Outcome criteria: no. of death to be prevented no of patients cured.
- Methodology: methodology of evaluation should be based on the purpose of evaluation. Gathering information: evaluation requires collection of data or information. The amount of data required will depend on the purpose and the use of the evaluation.
- Analysis of results: analysis of the data and interpretation of data and feedback to all individuals concerned should take place which will provide opportunity for discussing the evaluation results.
- Taking action: Based on the evolution results actions are taken to strengthen or modify the programme, which may call for shifting priorities, revising the objectives etc.
- Re-evaluation: Evaluation is an ongoing process which is needed to make health programmes more relevant, efficient and effective

Characteristics of a good plan

- Feasible-have resources to carry out plan
- Measurable – specific goals
- Time specific – comprehensive time lines, work plans, deadlines
- Flexible – scheduled review, revision
- Inspiring – worth working for
- In writing – plan clear to all

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Factors that may make plan fail e.g

i. Economy
ii. Unstable government
iii. Inconsistent policy
iv. Not evidence based
v. Non participation
vi. Wrong strategies/poor tactics
vii. No/poor forecasting
viii. Lack of commitment

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Public Health Programmes in Kenya

1. Reproductive and Child Health

The Ministry of Health has sanctioned the existence and free operation of the Reproductive Health and Child Health Divisions.

The Divisions have provided an annual report based on data from provincial, district, sub-district and rural health facilities.

Components of the reproductive health programmes

1. Safe motherhood, including antenatal, safe delivery, and postnatal care, especially breastfeeding, infant health and women's health;

2. Family planning;

3. Prevention and treatment of unsafe abortions and post-abortion care

4. Prevention and treatment of reproductive tract infections, including sexually transmitted diseases and HIV/AIDS;

5. Prevention and treatment of infertility;

6. Discouragement of harmful traditional practices that affect the reproductive health of men and women, such as female genital mutilation;

7. Information and counselling on human sexuality, responsible sexual behaviour, responsible parenthood, preconception care, and sexual health;

8. Gender and reproductive rights.

2. The Child Health Division

Constitutes all child health activities aimed at **promoting and maintaining the optimal growth and development** of children age 0-18 years

Specific responsibilities of Child Health division

- Ensure **survival, growth and development** of children age 0-5 years;
- Promote **health in all children**, pre-schooland school-age, including adolescents (up to 18 years), both in and out of school;

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- Promote **good nutrition** to children, expectant and nursing mothers, the sick, and the general population, including elimination of micronutrient deficiencies;
- Promotion **of child's rights** and child protection

Strategies applied to ensure quality child health

A. Early childhood:

1. Integrated Management of Childhood Illnesses (IMCI) 0-5 years

2. Comprehensive school health programme 6-18 years

B. Neonatal health care:

1. Nutrition programme;

2. Promotion of exclusive breastfeeding for six months and timely intervention of complementary feeding;

3. Immunisation;

4. Growth promotion and nutrition rehabilitation;

5. Curative care for minor ailments and injuries;

6. Promotion of family/household practices that have greatest impact on child health;

7. Rights promotion.

3. School health services:

1. Screening and examination of school children and food vendors;

2. Immunisation and micronutrient supplementation;

3. Health education on current public health issues;

4. Introduction of life skills and moral values including reproductive health;

5. Maintenance of a hygienic school environment;

6. School deworming;

4. Adolescent health:

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1. Referrals identification and management of common health problems affecting adolescents;

2. Provision of services focused on adolescents, including **counselling, information, education and communication** (IEC), and reproductive health issues in general;

Community Participation

Community participation can be loosely defined as the involvement of people in a community in projects to solve their own problems. People cannot be forced to 'participate' in projects which affect their lives but should be given the opportunity where possible. This is held to be a basic human right and a fundamental principle of democracy. Community participation is especially important in emergency sanitation programmes where people maybe unaccustomed to their surroundings and new sanitation facilities.

Community participation can take place during any of the following activities:

1. Needs assessment --expressing opinions about desirable improvements, prioritising goals and negotiating with agencies
2. Planning– formulating objectives, setting goals, criticising plans.
3. Mobilising– raising awareness in a community about needs, establishing or supporting organisational structures within the community.
4. Training – participation in formal or informal training activities to enhance communication, construction, maintenance and financial management skills
5. Implementing– engaging in management activities; contributing directly to construction, operation and maintenance with labour and materials; contributing cash towards costs, paying of services or membership fees of community organisations
6. Monitoring and evaluation – participating in the appraisal of work done, recognising improvements that can be made and redefining needs

Incentives of community participation

The following are some of the main reasons why people are usually willing to participate in humanitarian programmes:

1. Community participation motivates people to work together – people feel a sense of community and recognise the benefits of their involvement.

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2. Social, religious or traditional obligations for mutual help Genuine community participation – people see a genuine opportunity to better their own lives and for the community as a whole

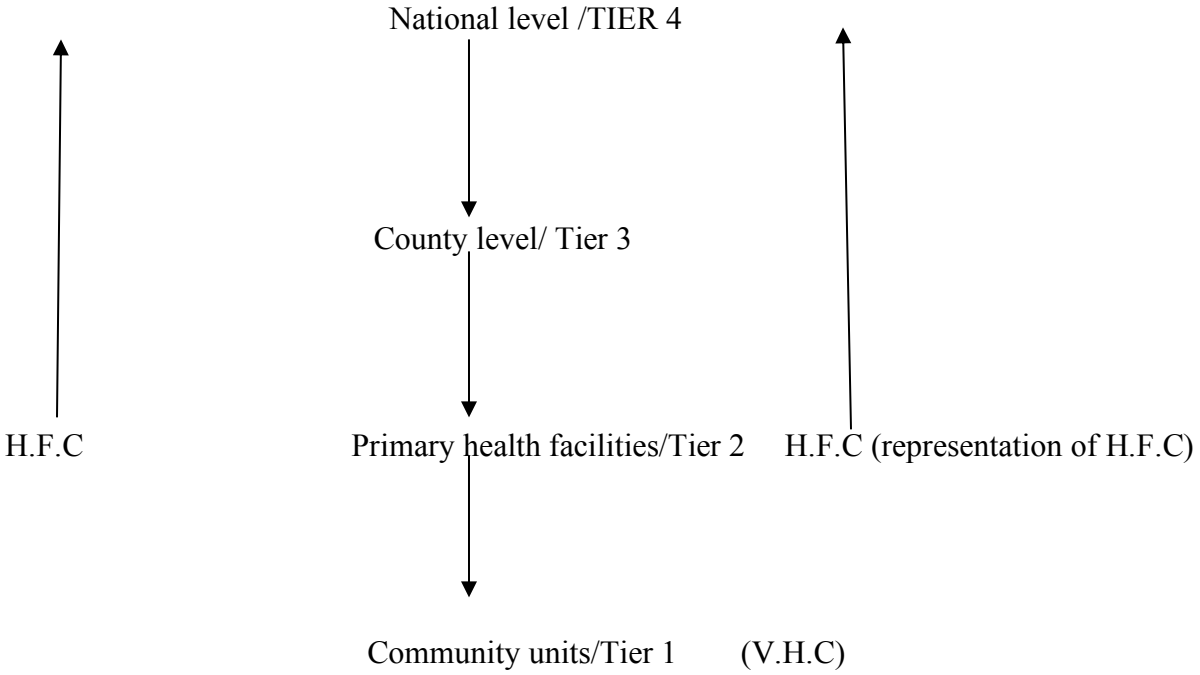
3. Remuneration in cash or kind ..

There are often strong genuine reasons why people wish to participate in programmes. All too often aid workers assume that people will only do anything for remuneration and have no genuine concern for their own predicament or that of the community as a whole. This is often the result of the actions of the agency itself, in throwing money or food at community

Members without meaningful dialogue or consultation. Remuneration is an acceptable incentive but is usually not the only, or even the primary, motivation

Management of Health facilities

These plans are then presented and negotiated at the village meeting, which brings all the plans together into one village plan.



VFC-----village facility committee

HFC----- health facility committee

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The intention of the decentralization policy is to enable the community to participate effectively in decision making processes related to matters of health at the community level, as well as at the interface between level 1 and levels 2 and 3. The lower levels of care (levels 2 and 3) are more or less consistent with the administrative and development nodes within the community.

Health facility management committees, made up of community representatives and the facility in-charges, should meet at least monthly.

The committees will review progress from the indicators and baselines generated through the CBIS and FBIS and make decisions for continued actions for health, at facility, community, household, political and administrative levels

Participation of communities in health management at tier 1

Community Health extension workers (CHEWs)

Community health extension workers (CHEWs) tend to be recruited by the health system and assigned to the local structures. Main roles include training and continued support for the CHWS according to the felt needs of the community. They include P.H.Technitian, P.H.Nurse, and P.H.Officer

The CHEWs are based at a health facility but assigned to work within a specific sub-location to ensure acceptable standards of care at level 1. They provide continuing training to CHWS through demonstration and instruction based on immediate learning needs. They thus train the CHWS on the job as they provide services at level

This is why the CHEWs are referred to as "coaches" of the CHWS. This is the essence of the community system

Their tasks will therefore include:

- Training CORPs using problem-based learning methods in service contexts.
- Implementing NHSSP III and the KEPH at level 1.
- Organizing and facilitating community animation and mobilization.

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- Carrying out evidence-based management of service delivery at level 1, grounded in a continuous improvement strategy.
- Supporting CORPs in recognition and classification of health problems and action to resolve them.
- Promoting inter-sector action for health, working with various extension workers.
- Facilitating health promotion through advocacy, social mobilization and interactive communication to improve key household practices for health.
- Functioning as link person between communities and the health system.
- Carrying out immunization, family planning, antenatal care, home delivery, disease surveillance, treatment of common conditions, prevention and control of HIV/AIDS, STI, TB, and promotion of school health.
- Managing resources, and storing and distributing commodities and supplies for CORPs.
- Mobilizing communities and their leaders on LEVEL ONE SERVICES to enable them to take appropriate action.
- Facilitating assessment, planning, implementation, monitoring and evaluation of LEVEL ONE SERVICES.
- Establishing and managing community- and facility-based information systems, to include data collation, storage, analysis, interpretation and utilization, in dialogue for continuous improvement.
- Carrying out baseline survey and analysing data.
- Keeping records of daily activities of services delivered and producing and submitting reports.
- Organizing documentation and filing system.

HOSPITAL MANAGEMENT

- **HOSPITALS ----**Formal institutions developed by the society for patient care Intended to meet the complex health needs of its members
- Individual-sick or injured has access to centralized medical knowledge & technology
- Society-it protects the family from many of the disruptive effects of caring for the ill in the home and making the problems less disruptive for the society as a whole

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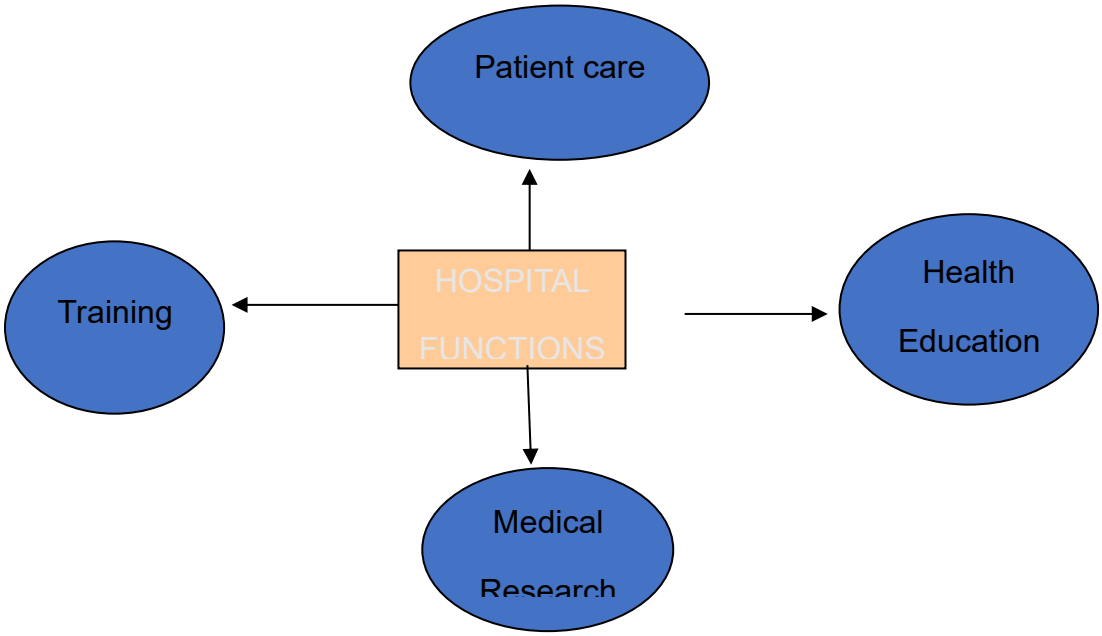
At Present

- Hospital is a place for the diagnosis and treatment of human ills and restoration of health and well-beings of those temporarily deprived of these.
- Professionally & technically skilled people apply their knowledge and skill with the help of complicated equipment and appliances - to provide quality care for the patient

Hospital functions



HOSPITAL ORGANIZATION

- A hospital is a multifaceted organization comprising many committees, departments, types of personnel, and services.
- It requires highly trained employees, efficient systems and controls, necessary supplies, adequate equipment and facilities, and, of course, physicians and patients

1. **Board of Trustees**

- The "board of trustees," or governing board, operates the hospital in trust for the community and has a fiduciary duty to protect the assets of the hospital through efficient operation.

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- The trustees are responsible for establishing the hospital's mission and establishing its bylaws and strategic policies.
- Trustees select the administrative leader of the hospital and delegate the hospital's daily operations and budgeting to the appointed executive.

2. **Executive Administration**
   - The chief executive officer (CEO) reports to the governing board and provides leadership in implementing the strategic goals and decisions set by the Board.
   - The CEO also represents the hospital to the external environment and the community.
   - In these tasks, the CEO must coordinate the collective effort of the hospital's personnel.

3. **The Medical Staff**

- The physician is the leader of the clinical team and the major agent working on behalf of the patient.
- The physician's responsibility is to diagnose the patient's condition accurately and to prescribe the best and most cost-effective treatment plan.
- The medical staff is a formally organized self-governing unit within the hospital, primarily comprised of physicians, but may also include other doctoral level health care professionals such as dentists or psychologists

4. **Nursing Services**
   - Nursing services employees are responsible for carrying out the treatment plan developed by the physician.
   - Nursing services, also called patient care services, is the largest component of the hospital.

5. **Allied Health Services**
   - A number of departments perform support functions that help with diagnosis and treatment.
   - The clinical laboratory is a diagnostic center that performs a variety of functions, including autopsy, clinical cytology, and clinical pathology
   - Also medical technologists, radiology department and rehabilitation services
6. **Clinical Support Services**

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The hospital pharmacy purchases and dispenses all the medications used to treat patients in the hospital.

The pharmacist works directly with the medical staff in establishing a formulary, the listing of drugs chosen to be included in the pharmacy

7. **Administrative Support Services**
    - Non-medical administrative services are necessary to the hospital's business and physical plant management.
    - The CEO leads these administrative services and is directly responsible for the day-to-day operations of the facility.
    - Business services manages the hospital's admitting and discharge functions, records charges to a patient's account, and handles accounts receivables with third-party payers such as insurance companies

CLASSIFICATION OF HOSPITALS

Hospitals can be classified according to the following

1. According to their OBJECTIVES

- Teaching-cum-Research Hospitals Eg: AIIMS
- General Hospitals – To provide medical care. Eg. District, Taluk, PHCs
- Special Hospitals – Concentrate on a particular aspect or organ of the body and provide medical care. Eg. Cancer, Dental, Psychiatry, T.B etc

2. On the basis of OWNERSHIP.

- Government hospitals
- Semi-Government hospitals
- Voluntary Agencies' hospitals
- Private / Charitable hospitals

3. According to the System of Medicine

- Allopathic hospitals
- Ayurvedic hospitals
- Homeopathic hospitals
- Unani hospitals

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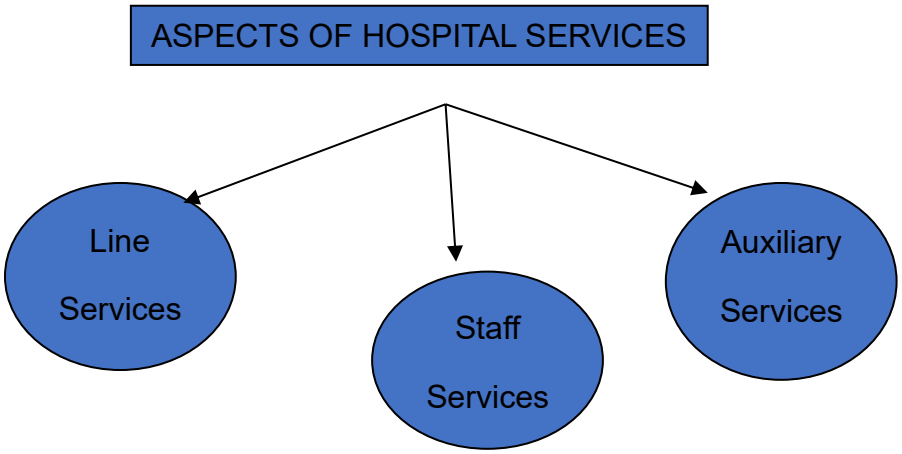
- Hospitals of other systems of medicine

4. According to Size

- Teaching Hospitals:- 500 beds
- District Hospitals:- 200 beds
- Taluk Hospitals:- 50 beds
- PHCs:- 6 beds

Components of Hospital system



Line Services

- Emergency services

  Diagnosis & treatment of illness of an urgent nature & injuries from accidents
- Out-Patient services

  Provision of diagnostic, curative, preventive and rehabilitative services
- In-patient services (Wards)
- Intensive care unit

  Those who need Acute, multidisciplinary and intensive observation and treatment
- Operation theatres

  Should have a pre-anaesthesia room and sterilisation room and a scrub room for doctors and nurses

Supportive (staff) services

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- Central sterile Supply Services Management
- Diet Management (Catering dept.)
- Pharmacy Services Management
- Laundry
- Laboratory facilities
- Radiology
- Nursing services

Auxiliary Services

- Registration and indoor case records
- Stores
- Transport
- Mortuary
- Dietary services
- Engineering and maintenance services
- Hospital security

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